

UNITED STATES PATENT AND TRADEMARK OFFICE

BEFORE THE PATENT TRIAL AND APPEAL BOARD

DR. REDDY'S LABORATORIES, INC.,
Petitioner,

v.

GENENTECH, INC., HOFFMAN-LA ROCHE AG, and ABBVIE INC.,
Patent Owner

PGR2022-00023
Patent 10,993,942 B2

Before JEFFREY N. FREDMAN, ZHENYU YANG, and ROBERT A.
POLLOCK, *Administrative Patent Judges*.

PER CURIAM.

Opinion Concurring-in-part and Dissenting-in-part filed by *Administrative
Patent Judge* FREDMAN.

JUDGMENT
Final Written Decision
Determining some challenged claims unpatentable
35 U.S.C. § 328(a)

I. INTRODUCTION

A. Background and Summary

Dr. Reddy's Laboratories, Inc. ("Petitioner") filed a Petition (Paper 2, "Pet.") requesting post-grant review of claims 1–3, 5, 6, 14, 25–27, and 30 (the "challenged claims") of U.S. Patent No. 10,993,942 B2 (Ex. 1001, "the '942 patent"). Genentech, Inc., Hoffman-La Roche AG, and Abbvie Inc. ("Patent Owner") filed a Preliminary Response. Paper 6 ("Prelim. Resp."). We determined, based on the information presented in the Petition and Preliminary Response, that the '942 patent was eligible for post-grant review, and that at least some of the challenged claims were more likely than not unpatentable at least based on Petitioner's challenge under 35 U.S.C. § 112 for a lack of written description support. Paper 8 ("Institution Decision" or "Inst. Dec."). Pursuant to 35 U.S.C. § 324, the Board instituted trial on July 25, 2022, of all challenged claims on the written description ground. *Id.*

Following institution, Patent Owner filed a Response to the Petition (Paper 24, "PO Resp."), Petitioner filed a Reply to Patent Owner's Response (Paper 33, "Reply"), and Patent Owner filed a Sur-reply (Paper 37, "Sur-reply"). Both Petitioner and Patent Owner filed various Objections to evidence (Papers 11, 12, 25, 35). An oral hearing was held on April 18, 2023, and a transcript has been entered into the record (Paper XX, "Tr.").

We have jurisdiction under 35 U.S.C. § 6. This Final Written Decision is issued pursuant to 35 U.S.C. § 328(a). Based on the record before us, we conclude that Petitioner has demonstrated by a preponderance of the evidence that claims 2, 3, 5, 14, 25–27, and 30 of the '942 patent are

unpatentable but has not demonstrated by a preponderance of the evidence that claims 1 and 6 of the '942 patent are unpatentable.

B. Real Parties-in-Interest

Petitioner states that the real parties-in-interest for Petitioner are Dr. Reddy's Laboratories Ltd. and Dr. Reddy's Laboratories, Inc. Pet. 2.

Patent Owner states that the real parties-in-interest are Genentech, Inc., Hoffmann-La Roche Inc., and AbbVie Inc., the assignees of record for U.S. Patent No. 10,993,942. Paper 4, 1.

C. Related Proceedings

Petitioner indicates that the '942 patent was asserted in *AbbVie Inc. v. Alembic Pharms., Ltd.*, No. 1-20-cv-01009 (D. Del.) and in *AbbVie Inc. v. Dr. Reddy's Labs., Ltd.*, No. 1-20-cv-00968 (D. Del.). Pet. 2; *see* Ex. 1003; Ex. 1004.

D. The '942 Patent (Ex. 1001)

The '942 patent states the "invention is directed to a combination therapy involving a type II anti-CD20 antibody and a selective Bcl-2 inhibitor for the treatment of a patient suffering from cancer, particularly a CD20-expressing cancer." Ex. 1001, 1:20–23. The '942 patent teaches that, in one embodiment, the anti-CD20 antibody may be the GA101 antibody known as obinutuzumab, and "the selective Bcl-2 inhibitor is GDC-0199." *Id.* at 4:6–7, 4:14–5:2.

The '942 patent teaches the treatment may comprise "administering to said human a GA101 antibody and/or GDC-0199 in multiple dosing cycles. In an embodiment, each dosing cycle of the multiple dosing cycle is for at least 1 week." *Id.* at 4:12–15. According to the '942 patent:

A dosing period as used herein is meant a period of time, during which each therapeutic agent has been[] administered at least once. A dosing cycle is usually about 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, or 30 days, and, in one embodiment, 6, 7, 8, 9, 10, 11, 12, 13, or 14 days, for example, 7 or 14 days.

In certain embodiments, a dosing period is a dosing cycle.

Ex. 1001, 17:29–38.

The '942 patent teaches that in “certain embodiments, methods are provided for the treatment of a cancer in a human in need thereof comprising administering to said human a GA101 antibody and/or GDC-0199 in multiple dosing cycles. In an embodiment, each dosing cycle of the multiple dosing cycle is for at least 1 week.” *Id.* at 4:10–15. The '942 patent teaches embodiments where “following the last dosing cycle of multiple dosing cycles, GDC-0199 alone can be administered to the human in the absence of the GA101 antibody.” *Id.* at 4:45–49.

The '942 patent teaches:

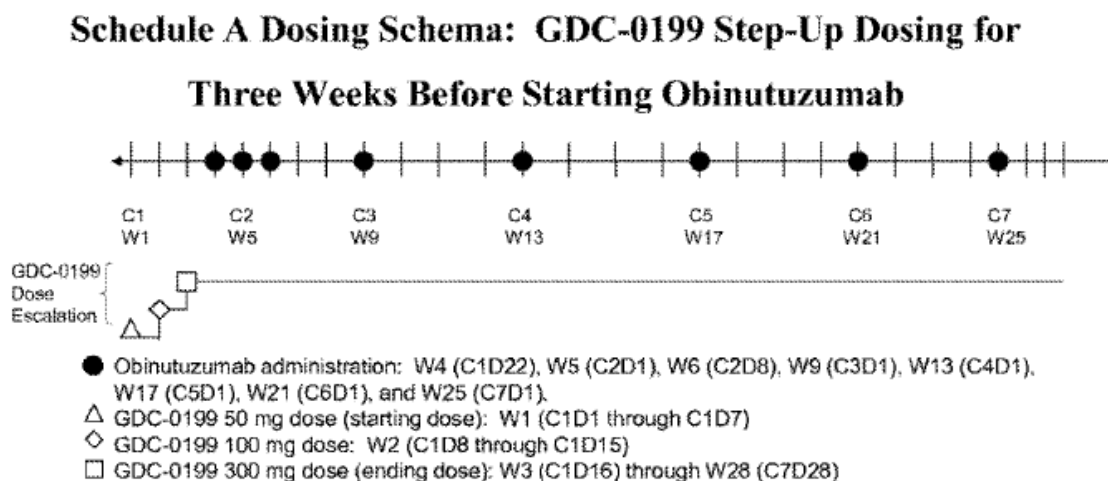
In certain embodiments, escalating doses of GDC-0199 are administered to the patient prior to administration of the type II anti-CD20 antibody. In other embodiments, escalating doses of GDC-0199 are administered to the patient after the type II anti-CD20 antibody has been administered to the patient.

In some embodiments of the method of treatment provided herein, the amount of GDC-0199 administered to the patient per dose is increased during the first dosing cycle from initial amounts of between 10 mg to 80 mg to final amounts of between 190 mg to 400 mg. In certain embodiments, the amount of GDC-0199 per dose administered to the patients begins with 50 mg or 100 mg, and is increased to 300 mg per dose. In some embodiments, the amount of GDC-0199 in the initial doses administered to the patient can, for example, be between 20 mg to 60 mg (e.g., 20 mg, 25 mg, 30 mg, 35 mg, 40 mg, 45 mg, 50

mg, 55 mg or 60 mg doses), followed by dose amounts of 100 mg, 200 mg, 300 mg or more of GDC-0199.

Ex. 1001, 24:42–60.

Figure 2 of the '942 patent provides an exemplary dosing schema and is reproduced below:



C = cycle; D = day; W = week.

Depicted above, “Schedule A (FIG. 2), consisting of GDC-0199 administered in escalating doses for 3 weeks prior to the first obinutuzumab infusion . . . the dose-finding stage will evaluate whether the administration of GDC-0199 prior to the first obinutuzumab infusion (Schedule A) will result in a lower incidence of infusion reactions.” *Id.* at 26:33–42.

The '942 patent teaches

GDC-0199 monotherapy may be continued in patients beyond 6-7 cycles of combination treatment described above (e.g., if they have acceptable toxicity and have not yet achieved maximal clinical response (i.e., are having continued improvement/reduction in tumor burden that has not yet stabilized for at least 2 months)). Such patients may continue GDC-0199 monotherapy until they have achieved maximal

response or up to 1 year after the last patient is enrolled,
whichever occurs first.

Ex. 1001, 26:59–67.

E. Illustrative Claims

The challenged claims are claims 1–3, 5, 6, 14, 25–27, and 30 of the
'942 patent, of which claims 1, 14, and 26 are the independent claims.

Claims 1, 2, and 14 are illustrative of the challenged claims and recite:

1. A method for treating chronic lymphocytic leukemia (CLL) or small lymphocytic lymphoma (SLL) in a patient, the method comprising orally administering GDC-0199 in escalating doses to the patient, wherein the escalating doses comprise a dose of 50 mg of GDC-0199 per day for one week followed by a dose of 100 mg of GDC-0199 per day for one week.
2. The method of claim 1, wherein the escalating doses further comprise a dose of 20 mg of GDC-0199 per day for one week.
14. A method for treating chronic lymphocytic leukemia (CLL) or small lymphocytic lymphoma (SLL) in a patient, the method comprising orally administering GDC-0199 in escalating doses to the patient, wherein the escalating doses comprise a dose of 20 mg per day for one week, a dose of 50 mg per day for one week, a dose of 100 mg per day for one week, a dose of 200 mg of GDC-0199 per day for one week, and a dose of 400 mg per day.

Ex. 1001, 35:43–53; 3:58–65.

F. The Instituted Ground of Unpatentability

We instituted trial based on the challenge to the patentability of
certain claims of the '942 patent presented in the Petition. Pet. 1–4, 6.

References	Basis	Claims Challenged
Written Description	§ 112(a)	1–3, 5, 6, 14, 25–27, 30

Petitioner relies on the Declaration of Jeffrey A. Gordon, M.D. *See* Ex. 1020. In support of its Response, Patent Owner relies upon the Declaration of Jeffrey P. Sharman, M.D. *See* Ex. 2008.

II. ANALYSIS

A. Level of Ordinary Skill in the Art

Petitioner contends that:

A person of ordinary skill in the art (“POSA”) in the field of the patent between 2012 and 2013, or in 2020, would have held an MD with experience in clinical oncology or pharmacology, a PhD in pharmacology, pharmaceutical chemistry, cancer biology, or a related discipline, or a PharmD with experience in clinical oncology. A person of ordinary skill would also have practical working knowledge of oncology. A POSA would also have had experience with the design of studies necessary for drug development. This description is approximate, and additional experience could make up for less education and vice versa.

Pet. 10–11 (citing Ex. 1020 ¶ 38).

Patent Owner contends “that a POSA for purposes of the ’942 patent would have had at least an M.D. and several years of experience treating patients with hematological malignancies (*i.e.*, blood cancers). Ex. 2008 ¶¶ 96, 98.” PO Resp. 16. Patent Owner contends that “[u]nlike Petitioner’s definition, Patent Owners’ definition properly accounts for the ’942 patent’s focus on the treatment of CLL/SLL by requiring experience with treating patients with hematological malignancies.” *Id.* at 17 (citing Ex. 2008 ¶ 96).

We do not discern a substantive difference between the parties’ respective definitions for the level of ordinary skill in the art. Although Petitioner did not expressly recite hematological malignancies, Petitioner did refer to expertise in clinical oncology and cancer biology, which would

encompass the blood cancers recited by Patent Owner. Accordingly, we find the parties' respective definitions to be equivalent and consistent with the level of ordinary skill in the art as reflected by the prior art in this proceeding. *See Okajima v. Bourdeau*, 261 F.3d 1350, 1355 (Fed. Cir. 2001) (explaining that specific findings regarding ordinary skill level are not required "where the prior art itself reflects an appropriate level and a need for testimony is not shown" (*quoting Litton Indus. Prods., Inc. v. Solid State Sys. Corp.*, 755 F.2d 158, 163 (Fed. Cir. 1985))).

We find that Dr. Gordon and Dr. Sharman are both qualified to opine from the perspective of a skilled artisan as both are persons of at least ordinary skill in the art, based on either party's definition. Ex. 1020, ¶¶ 8–19; Ex. 2008, ¶¶ 11–24.

B. Claim Construction

Petitioner contends that the preamble recitation to treat particular cancers is a limitation. *See* Pet. 22. Patent Owner contends that "[c]onstruing that term is unnecessary to resolve any controversy." Prelim. Resp. 14. Patent Owner also contends that the standard understanding of the transitional term comprising should be applied. *See id.* at 15–16.

The parties do not identify any issue relating to the preamble that needs to be addressed to resolve the sole written description challenge and the parties do not dispute that the transitional term "comprising" is "inclusive or open-ended and does not exclude additional, unrecited elements or method steps." *Georgia-Pacific Corp. v. US. Gypsum Co.*, 195 F.3d 1322, 1327 (Fed. Cir. 1999). We therefore find that there are no terms that need claim construction. *See, e.g., Wellman, Inc. v. Eastman Chem. Co.*, 642 F.3d 1355, 1361 (Fed. Cir. 2011) ("[C]laim terms need only be

construed ‘to the extent necessary to resolve the controversy.’”) (quoting *Vivid Techs., Inc. v. Am. Sci. & Eng’g, Inc.*, 200 F.3d 795, 803 (Fed. Cir. 1999)).

C. Principles of Law

The test for written description is “whether the disclosure of the application . . . reasonably conveys to those skilled in the art that the inventor had possession of the claimed subject matter as of the filing date.” *Ariad Pharms., Inc. v. Eli Lilly & Co.*, 598 F.3d 1336, 1351 (Fed. Cir. 2010) (en banc). “Based on that inquiry, the specification must describe an invention understandable to that skilled artisan and show that the inventor actually invented the invention claimed.” *Id.* “[W]hile the description requirement does not demand any particular form of disclosure, or that the specification recite the claimed invention *in haec verba*, a description that merely renders the invention obvious does not satisfy the requirement.” *Id.* at 1352 (citations omitted). “[T]he test requires an objective inquiry into the four corners of the specification from the perspective of a person of ordinary skill in the art.” *Id.* at 1351.

We analyze the asserted ground of unpatentability in accordance with the above-stated principles.

D. Written Description—Claim 1

1. Petitioner’s position

Petitioner asserts “the claims lack written description support in the provisional, which is missing much content from the ’942 patent’s specification.” Pet. 10. Petitioner asserts that:

The ’942 specification describes a combination therapy by administering *two* compounds, but its claims are directed to a

monotherapy only requiring *one*. The patent’s specification does not describe the claimed escalating-dose GDC-0199 monotherapy. Indeed, the ’942 patent’s title calls the invention a “combination therapy” of a Bcl-2 inhibitor (e.g., GDC-0199) *and* an antibody. So does its summary, its abstract, and its embodiments. So too do all its examples and figures. Further, the specification focuses on not just *any* antibody, but an *anti-CD20* antibody—and not just any anti-CD20 antibody, but the *type II* variety.

Id. at 5. Petitioner also asserts the ’942 patent claims “a specific *combination* of GDC-0199 dose amounts, times, duration, and sequence—a particular escalating-dose GDC-0199 monotherapy—that was not described in its specification. Nor is there guidance in the specification to select these parameters from a laundry list of options.” *Id.* at 6. Petitioner asserts “before the generics were sued, Patent Owner filed the application leading to the ’942 patent as a continuation of the ’595 application. This was the first time in the priority chain that a GDC-0199 monotherapy was claimed. . . .” *Id.* at 18.

Petitioner asserts “Patent Owners and their expert failed to account for numerous neon-bright signs in the specification—e.g., that ‘the invention’ is limited to combination therapies, and that the experiments do not disclose GDC-0199 alone or escalating doses as treatment methods.” Pet. Reply 3 (citing Pet. 29–42). Petitioner asserts “[t]he inquiry is whether a POSA would recognize that Patent Owners had invented an effective dose regimen. The answer is no. Petitioner’s evidence established that a POSA would not recognize which combinations of doses were effective.” *Id.* at 6. Petitioner asserts the “patent fails to describe any antibody-free monotherapy treatment, much less escalating-dose monotherapy.” *Id.* at 10.

Petitioner asserts

Patent Owners and their expert do not rebut Dr. Gordon's view that a POSA would not consider one dosing cycle in a larger treatment plan to constitute the whole method of treatment. *See, e.g.,* Ex. 1020 ¶¶ 120-21. This is precisely what Example 2 teaches: "[t]wo schedules ... consisting of GDC-0199 administered in escalating doses for 3 weeks *prior to the first obinutuzumab infusion*, and ... consisting of *obinutuzumab administered first* followed by escalating dose levels of GDC-0199." Ex. 1001, 26:31-37 (emphasis added); *see also id.* at 26:49-58 (calling both Example 2 schedules a "combination treatment").

Id. at 10–11. Petitioner asserts that although "Patent Owners tout isolated references to 'GDC-0199 alone' or 'a GA101 antibody *and/or* GDC-0199' as adequate support for the claims, POR 22-23, this is wrong. The patent plainly says, as Dr. Gordon pointed out, that these references are specific dosing cycles *within* a broader method of treatment." *Id.* at 11.

Petitioner asserts

Patent Owners cannot point to an example demonstrating that the inventors possessed at filing the later-claimed method for treating both CLL and SLL using a 50 mg daily dose for one week followed by a 100 mg dose for another week. The full scope of claim 1 requires description of a therapy with *only* these two doses, even if other doses can be present.

Pet. Reply 15. Petitioner asserts "Example 2 is a research plan, not an example of actual treatment: it is a hypothetical dose-finding Phase I clinical trial that would evaluate the dosing schemes in Schedules A/B (Figures 2/3)." Pet. Reply 16. Petitioner asserts "claim 1's two-dose regimen 1 is not Example 2's three-dose regimen. Although claim 1 allows additional doses, its full scope requires supporting at least its two-dose-*only* regimen, in which 100 mg is a final dose." Pet. Reply 16. Petitioner asserts "[n]othing in Example 2 (or elsewhere) suggests a reason to use GDC-0199 alone to treat

SLL and CLL.” Pet. Reply 17. Petitioner asserts “claim 1 covers treating *both* SLL and CLL. To adequately describe that scope, the specification had to disclose treating both diseases with the claimed method.” Pet. Reply 18.

2. Patent Owner’s position

Patent Owner asserts that “the challenged claims encompass both monotherapy and combination therapy, and the ’942 patent specification provides sufficient written description support for both. Institution Decision at 12-13. Dr. Sharman agrees. Ex. 2008 ¶¶ 101-121.” PO Resp. 21. Patent Owner asserts

the challenged claims recite methods of treatment “comprising” the administration of GDC-0199. It is well-settled law that “the transitional term ‘comprising’ . . . is ‘inclusive or open-ended and does not exclude additional, unrecited elements or method steps.’” See Institution Decision at 13 (citing *Georgia-Pacific*, 195 F.3d at 1327). Here, the use of the term “comprising” establishes that the claims encompass both the administration of GDC-0199 alone, or in combination with other therapeutic agents.

PO Resp. 21–22. Patent Owner asserts “[e]xamples 1, 3, and 4 of the specification show that monotherapy with GDC-0199 is efficacious.

Ex. 2008 ¶¶ 108-117. Example 1 tested the anti-tumor activity of GDC-0199 administered alone, obinutuzumab (a GA101 antibody) administered alone, and their combination.” PO Resp. 23. Patent Owner asserts as “compared to a placebo, GDC-0199 administered alone for 3 weeks ‘demonstrated stasis followed by tumor growth delay’ and showed 80% tumor growth inhibition. Ex. 1001, 25:65-67, 26:4-7; *see also* Ex. 2008 ¶¶ 60, 82, 110.” PO Resp. 23. Patent Owner asserts “[a]s Dr. Sharman opines, from the results of Example 1, as illustrated in Figure 1, a POSA would have recognized that GDC-0199 has independent activity in inhibiting tumor growth. Ex. 2008 ¶¶ 59, 111.”

PO Resp. 23. Patent Owner asserts that Figure 2 “instructs that GDC-0199 is administered at a dose of 50 mg per day for the first week, followed by 100 mg per day for the second week—the dosing recited in claim 1. Ex. 1001, Fig. 2; *see also id.*, 24:34-44.” PO Resp. 26.

Patent Owner asserts

as Dr. Sharman explains, CLL and SLL both result from the proliferation of malignant B-cells, are often referred to together as CLL/SLL, and are treated by hematologic oncologists, i.e., specialists in the treatment of blood cancers. *See* Ex. 2008, ¶ 42. Petitioner, however, would have Dr. Sharman not read the specification as a whole, and not bring the knowledge and skill of a POSA to his reading of the specification, as is proper for the written description inquiry.

PO Sur-reply 5. Patent Owner also asserts that Dr. Sharman

squarely refuted Dr. Gordon’s opinions, explaining that ‘Dr. Gordon’s assertion that the invention of the ’942 patent is limited to combination therapy is inconsistent with the plain text of the ’942 patent, which expressly discloses GDC-0199 monotherapy . . . as well as how a POSA would understand the disclosure.’ Ex. 2008, ¶ 107 (citing Ex. 1020 ¶¶ 113-117); *see also id.*, ¶¶ 114-117. Not only was Dr. Gordon’s opinion rebutted, it is contrary to the express disclosures of the ’942 patent.

PO Sur-Reply 9. Patent Owner asserts that “[n]owhere in the ’942 patent do Patent Owners disclaim GDC-0199 monotherapy.” PO Sur-Reply 10. Patent Owner asserts “[a]t bottom, Petitioner’s arguments regarding claim 1 are simply an effort to detract from the express disclosures of the specification.” PO Sur-Reply 13. Patent Owner asserts “Example 2 explicitly discloses administering a 50 mg daily dose of GDC-0199 for a week to previously untreated CLL patients, followed by a 100 mg daily dose of GDC-0199 for a week, as claimed in claim 1.” PO Sur-Reply 13–14.

3. *Analysis*

To prevail in this post-grant review of the challenged claims, Petitioner must prove unpatentability by a preponderance of the evidence. 35 U.S.C. § 326(e); 37 C.F.R. § 42.1(d). The petitioner has the burden from the onset to show with particularity why the challenged claims are unpatentable. *See Harmonic Inc. v. Avid Tech., Inc.*, 815 F.3d 1356, 1363 (Fed. Cir. 2016). This burden of persuasion never shifts to the patent owner. *See Dynamic Drinkware, LLC v. Nat'l Graphics, Inc.*, 800 F.3d 1375, 1378 (Fed. Cir. 2015) (discussing the burden of proof in inter partes review).

a. Experts' views of descriptive support for claim 1 in '942 patent

Dr. Gordon¹ challenges descriptive support for claim 1 in the '942 patent, stating “even if there are points where the specification mentions administering a dose of just GDC-0199, it is disclosing doing so as part of a combination therapy.” Ex. 1020 ¶ 69. Dr. Gordon does acknowledge that “within a co-administration method of treatment, individual dosing cycles might be one compound or the other” but asserts “that the whole method of

¹ We note that in the Gordon Deposition, Dr. Gordon acknowledged authorship of only two articles in the field of hematological oncology. Ex. 2030, 45:12–14. Dr. Gordon did not recollect when asked “Have you used murine models to evaluate promising candidates for therapeutic use?” Ex. 2030, 54:22–24. Dr. Gordon was unable to answer specific questions regarding the eleven articles on his CV without reviewing the articles and was unable to confirm his research experience, Ex. 2030:57:3–13. In response to a question “What is your role in these two [clinical] trials”, Dr. Gordon was unable to recollect. Ex. 2030, 66:4–6. When asked “Are you a principal investigator on this trial?” and “Did you have any responsibility for designing the [clinical] trial?” Dr. Gordon answered “I do not recollect.” Ex. 2030, 66:10–18.

treatment uses both.” Ex. 1020 ¶ 76. Dr. Gordon states “[a]t no point does the specification include every element of the claim (GDC-0199 monotherapy, as well as dose amount, timing, and sequence) in a single example.” Ex. 1020 ¶ 131.²

Dr. Sharman³ supports a finding of descriptive support in the ’942 patent, explaining that

in the case of CLL/SLL, historically they were thought of as separate diseases, but as we gained more understanding scientifically of these diseases, we realized it’s really just one manifestation of the same -- excuse me -- two manifestations of the same disease, so it’s commonplace in the literature to refer to CLL, slash, SLL as a single entity.

Ex. 1034, 18:5–11. Dr. Sharman stated that a proposed trial in the ’942 patent disclosed that an “exemplary dose escalation for treatment of patients with CLL is illustrated in Figure 2 of the patent, which . . . shows a dose escalation where GDC-0199 is administered at a starting daily dose of 50 mg per day for the first week, followed by a daily intermediate dose of 100 mg per day for the second week.” Ex. 2008 ¶¶ 69–70. In the deposition, Dr. Sharma stated that “GDC-0199 is given as monotherapy from cycle 1

² We note that Dr. Gordon does not specifically address the disclosure in the example shown in Figure 2 relative to the recitations of claim 1, but rather asserts that this example “discusses a combination therapy.” Ex. 1020 ¶ 124.

³ We note that in the Sharman Deposition (Ex. 1034), Dr. Sharman acknowledged involvement in the Genetech study which compared “venetoclax to chemotherapy, and in the context of leading that study as the principal investigator, I have provided feedback on study design and oversight.” Ex. 1034, 23:9–12. Dr. Sharman stated “I have helped with the development of several additional therapeutic drugs for Genentech, so I have had similar roles on other molecules.” Ex. 1034, 24:17–19.

day 1 all the way to the completion of therapy at week 28, which is cycle 7 day 28.” Ex. 1034, 87:16–19.

We find Dr. Sharman’s statements and deposition testimony have substantially greater credibility than Dr. Gordon’s statements and deposition testimony based on Dr. Sharman’s significantly greater relevant experience in treatment and study of hematological cancers using chemotherapeutic agents and based on Dr. Sharman’s superior recollection and more complete analysis of the record found in the ’942 patent. In particular, we found Dr. Gordon’s statement that no example “includes” every element of claim 1 factually incorrect as contradicted by at least Figure 2 of the ’942 patent where the ordinary understanding of includes to mean encompasses is used. *See* Ex. 1020 ¶ 131. *Ebit Sys. Am., LLC v. Thales Visionix, Inc.*, 881 F.3d 1354, 1358 (Fed. Cir. 2018) (“The PTAB [i]s entitled to weigh the credibility of the witnesses”).

b. Descriptive support analyzed in view of intrinsic ’942 Specification, expert declarations, and case law

It is well-settled that the “transition ‘comprising’ in a method claim indicates that the claim is open-ended and allows for additional steps.” *Invitrogen Corp. v. Biocrest Mfg., L.P.*, 327 F.3d 1364, 1368 (Fed. Cir. 2003). Likewise, “[t]he statutes permit, and it is the settled practice of the Patent Office, many times sustained by this Court, to allow claims to a combination and also its subcombinations.” *Special Equipment Co. v. Coe*, 324 U.S. 370, 377 (1945).

We find that Figure 2 of the ’942 patent and the associated example provide literal and express descriptive support for claim 1 of the ’942 patent. As descriptive support for claim 1, the ’942 patent teaches treatment of

leukemia or lymphoma, including chronic lymphocytic leukemia and small lymphocytic lymphoma (Ex. 1001, 16:21–22), by orally administering the Bcl-2 inhibitor GDC-0199 (Ex. 1001, 17:16) in escalating doses of 50 mg per day during a first week and 100 mg per day during a second week (Ex. 1001, Figure 2).

Claim 1 of the '942 patent, as written, does not exclude performance of the complete combination therapy shown in Figure 2. Rather, the reasonable understanding of the term “comprising” in claim 1 encompasses monotherapy with GDC-0199 alone and combination therapy with obinutuzumab. Both embodiments are described in the Specification. As Patent Owner points out, the '942 Specification is replete with disclosures to administer GDC-0199 by itself to a patient, stating

“methods are provided for the treatment of a cancer in a human in need thereof comprising administering to said human a GA101 antibody and/or GDC 0199 in multiple dosing cycles”; “GDC-0199 alone can be administered to the human in the absence of the GA101 antibody”; “[p]harmaceutical compositions of the anti-BCL-2 active agent alone, e.g., the Bcl-2 inhibitor”; “In certain dosing cycles, one therapeutic agent alone is administered to the patient”; “the patient can then be administered with GDC-0199 alone (e.g. in a maintenance phase)”; “escalating doses of GDC-0199 are administered to the patient prior to administration of the type II anti-CD20 antibody”; “GDC-0199 monotherapy may be continued in patients beyond 6–7 cycles of combination treatment described above . . . Such patients may continue GDC-0199 monotherapy until they have achieved maximal response.”

See, e.g., PO Resp. 8, 9, 12, 22, 47 (citing Ex. 1001, 4:10–13; 4:46–48; 22:5–6; 24:2–4; 24:16–17; 24:42–44; 26:58–66.) Dr. Sharman states “a POSA would have recognized from Example 1 and Figure 1 . . . that both GDC-0199 combination therapy and monotherapy were effective methods of treating CLL and SLL.” Ex. 2008 ¶ 111.

As discussed in the Institution Decision, we find Petitioner’s argument (*see* Pet. 25–26) relating to a lack of support in US provisional application 61/698,379 irrelevant because the issue is not whether the claims have priority sufficient to defeat intervening prior art but rather whether the claims as issued “overreach the scope of the inventor’s contribution to the field of art as described in the patent specification.” *Ariad*, 598 F.3d at 1353. These claims are fully described in the ’942 patent as discussed above.

As discussed in the Institution Decision, we find Petitioner’s argument purporting to limit the disclosure of the ’942 patent solely to combination therapies directly inconsistent with the plain text in the ’942 patent. *See* Pet. 29–30. Petitioner reiterates these arguments, contending that “Patent Owners and their expert failed to account for numerous neon-bright signs in the specification—e.g., that ‘the invention’ is limited to combination therapies, and that the experiments do not disclose GDC-0199 alone or escalating doses as treatment methods.” Pet. Reply 3.

We find this argument unpersuasive because the ’942 patent recognizes “GDC-0199 alone can be administered to the human in the absence of the GA101 antibody, or the GA101 antibody alone can be administered to the patient in the absence of GDC-0199.” Ex. 1001, 4:46–49. That the ’942 patent teaches alternatives of administering a GA101 antibody and/or GDC-0199 in multiple dosing cycles demonstrates possession by the ’942 patent of three alternative treatments: GA101 antibody and GDC-0199 together; GA101 antibody alone; and GDC-0199 alone. Ex. 1001, 4:10–15.

We also agree with Patent Owner that claim 1 encompasses therapies with GDC-0199 alone because claim 1 uses the transitional term “comprising” that is “inclusive or open-ended and does not exclude additional, unrecited elements or method steps.” *Georgia-Pacific Corp. v. US. Gypsum Co.*, 195 F.3d 1322, 1327 (Fed. Cir. 1999); *see* Prelim. Resp. 15. Claim 1 also does not exclude combination treatment with the GA101 antibody in addition to the recited dose escalation with GDC-0199.

Petitioner contends “Patent Owners and their expert do not rebut Dr. Gordon’s view that a POSA would not consider one dosing cycle in a larger treatment plan to constitute the whole method of treatment. *See, e.g.*, Ex. 1020 ¶¶ 120-21.” Pet. Reply 10. Petitioner points to *LizardTech, Inc. v. Earth Resource Mapping, Inc.*, 424 F.3d 1336, 1340–46 (Fed. Cir. 2005) and contends

[t]he only difference between *LizardTech*’s claim 1 (written description deemed adequate) and claim 21 (written description deemed inadequate) was that the latter omitted a few essential steps. *Id.* at 1340–47. If the open-ended “comprising” claim form were controlling, the Court would have determined that claim 21 was adequately described because its scope would include the narrower, adequately described claim 1 and related examples. Instead, the Court relied on the alleged advantages of the invention, the prosecution history, and statements by the patentee about “the invention claimed by applicant as a whole” in concluding that claim 21 was too broad without the omitted steps. *Id.* at 1343–45.

Paper 45, 1 (emphasis added).

We find this argument unpersuasive as *Lizardtech* explains that it is unnecessary to spell out every detail of the invention in the specification; only enough must be included to convince a person of skill in the art that the inventor possessed the invention and to enable such a person to make and use the invention without

undue experimentation. In this case, however, LizardTech has failed to meet either requirement. After reading the patent, a person of skill in the art would not understand how to make a seamless DWT generically and would not understand LizardTech to have invented a method for making a seamless DWT, except by “maintaining updating sums of DWT coefficients.”

Lizardtech, 424 F.3d at 1345. The *Lizardtech* facts differ from those of claim 1 of the '942 patent because, as supported by Dr. Sharman, the '942 patent both describes and enables the ordinary artisan to treat CLL/SLL with GDC-0199 in an escalating dose series as described in Figure 2 with a reasonable expectation of success. Ex 1001, Figure 2; Ex 2008 ¶¶ 59, 71, 111. For example, Dr. Sharman notes that Figure 4 of the '942 patent “shows that GDC-0199 treatment alone shows antitumor activity (tumor growth inhibition of 59%) compared to the vehicle alone.” Ex. 2008 ¶ 84.

Dr. Sharman states a “POSA also would have understood this to be evidence of efficacy of both monotherapy and combination therapy.” Ex. 2008 ¶ 85.

Moreover, *Lizardtech* compared their situation to *Tronzo v. Biomet, Inc.*, 156 F.3d 1154, 1159 (Fed. Cir. 1998) where *Tronzo* found “there was nothing in the patent’s specification ‘to suggest that shapes other than conical are necessarily a part of the disclosure.’” *Lizardtech* at 1346. That factual determination is very different from claim 1 of the '942 patent where there is no doubt that Figure 2 of the '942 patent exemplifies administration of GDC-0199 alone to chronic lymphocytic leukemia patients in escalating doses with 50 mg per day during a first week and 100 mg per day during a second week as required by claim 1 of the '942 patent. Ex. 1001, Figure 2; Ex. 1001, 26:27–35.

Petitioner's reliance on *PIN/NIP, Inc. v. Platte Chemical Co.*, 304 F.3d 1235, 1247–48 (Fed. Cir. 2002) is also unavailing. Paper 45, 2. In *PIN/NIP*, the issue was that two compounds, known in the prior art, unexpectedly suppressed tuber sprouts when used in combination. *Id.* at 1244. *PIN/NIP* therefore interpreted the term “composition” to mean mixture of both components. This is unlike claim 1 of the '942 patent where there is no evidence of record that monotherapy as recited in claim 1 would have been anticipated or obvious by the prior art. Moreover, unlike in *PIN/NIP* where the “described invention is clear; it is a mixture of the two chemicals together” (*id.* at 1244), the '942 patent provides express disclosures to use monotherapy with GDC-0199 alone as discussed above.

Claim 1 need not include every embodiment in the disclosure of the '942 patent so long as there is clear possession of the claimed invention. “That claims are interpreted in light of the specification does not mean that everything expressed in the specification must be read into all the claims.” *Raytheon Co. v. Roper Corp.*, 724 F.2d 951, 957 (Fed. Cir. 1983).

We therefore do not read the combination therapy into claim 1 when the '942 patent discloses that GDC-0199 alone may be administered for some period of time. Ex. 1001, Figure 2. *See, e.g., ScriptPro LLC v. Innovation Assocs., Inc.*, 833 F.3d 1336, 1341 (Fed. Cir. 2016) (“[A] specification's focus on one particular embodiment or purpose cannot limit the described invention where that specification expressly contemplates other embodiments or purposes.”).

Indeed, “[e]ven when the specification describes only a single embodiment, the claims of the patent will not be read restrictively unless the patentee has demonstrated a clear intention to limit the claim scope using

‘words or expressions of manifest exclusion or restriction.’” *Liebel-Flarsheim Co. v. Medrad, Inc.*, 358 F.3d 898, 906 (Fed. Cir. 2004) (quoting *Teleflex, Inc. v. Ficoso N. Am. Corp.*, 299 F.3d 1313, 1327 (Fed. Cir. 2002))). As the evidence shows, the ’942 patent expressly recites “administering to said human a GA101 antibody **and/or** GDC-0199 in multiple dosing cycles.” Ex. 1001, 4:10–15 (emphasis added). Similarly, the ’942 patent teaches “GDC-0199 alone can be administered to the human in the absence of the GA101 antibody.” Ex. 1001, 4:45–49.

We also find unpersuasive Petitioner’s argument that, “Example 2 is a research plan, not an example of actual treatment: it is a hypothetical dose-finding Phase I clinical trial that would evaluate the dosing schemes in Schedules A/B (Figures 2/3).” Pet. Reply 16. And such “a ‘mere wish or plan’ is not adequate written description.” *Id.* (citing *Centocor Ortho Biotech, Inc. v. Abbott Labs.*, 636 F.3d 1341, 1348 (Fed. Cir. 2011)).

“It is well established that a patent may be secured, and typically is secured, before the conclusion of clinical trials.” *In re Montgomery*, 677 F.3d 1375, 1382 (Fed. Cir. 2012). The clinical trial protocol in Figure 2 of the ’942 patent is clearly a disclosure that describes the invention. *See, e.g., Montgomery*, 677 F.3d at 1382 (“[E]ven if HOPE [clinical trial] merely proposed administering ramipril for treatment or prevention of stroke (without actually doing so), it would still anticipate.”). And Petitioner’s reliance on *Centocor* is misplaced, as in *Centocor* the concern was that the claims recited functional humanized antibodies with particular specificity but the Specification at issue did not “disclose any fully-human, high affinity, neutralizing, A2 specific antibody.” *Centocor*, 636 F.3d at 1349. The ’942 Specification provides specific details of the particular therapy

being used, including the particular compound GDC-0199, an example of a particular dose escalation of GDC-0199 in a particular patient population. *See* Ex. 1001, Figure 2.

As discussed in the Institution Decision, we find Petitioner’s argument that “Patent Owner claimed a specific *combination* of GDC-0199 dose amounts, times, duration, and sequence—a particular escalating-dose GDC-0199 monotherapy—that was not described in its specification,” incorrect with respect to independent claim 1, based on the complete record developed during trial. *See* Pet. 6. Figure 2 of the ’942 patent shows administration of GDC-0199 alone to chronic lymphocytic leukemia patients in escalating doses with 50 mg per day during a first week and 100 mg per day during a second week as required by claim 1 of the ’942 patent. Ex. 1001, Figure 2; Ex. 1001, 26:27–35.

Also as discussed in the Institution Decision, we find irrelevant Petitioner’s argument that Patent Owner filed this application because they were motivated by competition with generics. *See* Pet. 18. “[I]t is not improper for an applicant to broaden his claims during prosecution in order to encompass a competitor’s products, as long as the disclosure supports the broadened claims.” *See Liebel–Flarsheim*, 358 F.3d at 909 n.2.

We remain unpersuaded that the case law cited in the Petition supports Petitioner’s written description position on claim 1. As Petitioner notes, the issue in *Gentry Gallery* was that “the specification itself only described sofas ‘having controls on the console.’” Pet. 29 (citing *Gentry Gallery, Inc. v. Berkline Corp.*, 134 F.3d 1473, 1478–79 (Fed. Cir. 1998)). However, the ’942 patent does not limit the treatment method to a single embodiment, but expressly recognizes “GDC-0199 alone can be

administered to the human in the absence of the GA101 antibody.”

Ex. 1001, 4:45–49. Thus, the instant fact pattern differs from *Gentry Gallery* because, here, there is express disclosure of the claimed embodiment, and that embodiment is also disclosed in an example.

Ex. 1001, Figure 2; Ex. 1001, 26:27–35.

We also find Petitioner’s reliance on *ICU Medical* unavailing as there “the specification describe[d] only medical valves with spikes.” *ICU Medical, Inc. v. Alaris Medical Systems, Inc.*, 558 F.3d 1368, 1378 (Fed. Cir. 2009). In the present case, the ’942 patent expressly describes that “GDC-0199 alone can be administered to the human in the absence of the GA101 antibody.” Ex. 1001, 4:45–49. Moreover, the ’942 patent exemplifies a situation where GDC-0199 is administered alone for three weeks. Ex. 1001, Figure 2; Ex. 1001, 26:27–35.

Accordingly, we find that Petitioner does not show, by a preponderance of the evidence, that claim 1 of the ’942 patent is unpatentable for failing to comply with the written description requirement.

E. Written Description—Claim 2

Claim 2 depends from claim 1 and further recites “wherein the escalating doses further comprise a dose of 20 mg of GDC-0199 per day for one week.”

1. Petitioner’s Position

Petitioner contends the “claims recite a precisely picked array of escalating doses: 50 mg and 100 mg in claim 1, adding a 20 mg cycle in claim 2, 200 mg in claim 3, and 400 mg in claim 5—each for ‘one week.’” Pet. 43. Petitioner contends that

[w]here these specific numbers are mentioned in the specification, they appear in the context of a summary of “an embodiment wherein the GA101 antibody and GDC-0199 are administered to the human in multiple dosing cycles.” Ex. 1001, 4:26-37; *see also id.* at 24:48-60; Ex. 1020 ¶ 124. And even there, nothing in those passages provides guidance, within an escalating-dose regimen, that each dose be given for “one week.” Ex. 1020 ¶ 124.

Pet. 43. Petitioner further contends “[e]ven where the phrase ‘escalating doses’ appears, it presents as a laundry list, not a teaching.” Pet. 44.

Petitioner also contends

Patent Owner might argue that a skilled artisan *could* combine these pieces. But that possibility is not enough. The combination must be described. *See Biogen*, 18 F.4th at 1342-45 (rejecting written description despite each claim element being *individually* disclosed in the specification because the “skilled artisan would not have recognized” that the specific elements would be linked together); *LizardTech, Inc. v. Earth Res. Mapping, Inc.*, 424 F.3d 1336, 1346 (Fed. Cir. 2005) (explaining that it is not enough that each individual element is described)

Pet. 47. Petitioner contends Patent Owner doesn’t explain “why a POSA would do daily dosing instead of weekly. Nor could they, because the patent does not explain what step-up dosing accomplished—much less how or why to tweak it. . . . That amounts to a hindsight-driven attempt to patch together combinations from an amalgam of disclosures.” Pet. Reply 20–21.

2. Patent Owner’s Position

Patent Owner contends “Claim 2 recites daily administration of 20 mg/day for one week before the two weeks of treatment recited in independent claim 1 (50 mg/day, then 100 mg/day).” PO Resp. 28. Patent Owner contends that

[c]olumn 24 discloses that “the amounts of GDC-0199 per dose administered to the patient are increased during a first dosing cycle.” Ex. 1001, 24:34-37. Column 24 then refers to the schedule of Example 2 and Figure 2 as an example of this step-up dosing. *Id.*, 24:37-41 (“See, e.g., FIG. 2 for an exemplary dosing scheme . . .”). Column 24 goes on to disclose a small number of alternative dose amounts.

PO Resp. 30. Patent Owner contends “column 24 discloses only seven exemplary “initial doses” below 50 mg, one of which is the 20 mg initial dose recited in claim 2. Ex. 2008 ¶ 144.” PO Resp. 32. Patent Owner contends that,

[a]s Dr. Sharman explains, in view of the framework established in Example 2 and Figure 2, and the alternative dosing described in column 24 for use in that framework, a POSA would have readily understood that the inventors were in possession of the claimed method of treating CLL/SLL patients with the recited dose escalation of GDC-0199. *Id.* ¶ 147.

PO Resp. 32.

3. *Analysis*

a. Experts’ views of descriptive support for claim 2 in ‘942 patent

Dr. Gordon states “[a]t no point does the specification include every element of the claim (GDC-0199 monotherapy, as well as dose amount, timing, and sequence) in a single example.” Ex. 1020 ¶ 131. Dr. Gordon states “the physician would need to decide on doses, for some reason picking from 20, 50, 100, 200, and 400 mg as claimed, while ignoring all the others mentioned in certain embodiments at various points in the specification.” Ex. 1020 ¶ 134. Dr. Gordon states “[a]s a POSA, I see no disclosure why one would pick these doses, in this order, for these time periods, and all in

the absence of an antibody, and for these particular claimed cancers.”

Ex. 1020 ¶ 135.

Dr. Gordon, was asked “if you focus on doses -- initial doses lower than 50 milligrams, there's only seven examples, right?” Ex. 2030, 163:20–22. Dr. Gordon answered “No. I count -- lines 57 to 58 they're talking about, for example, 20 milligrams, 25, 30, 35, 40, 45. That’s one, two, three, four, five – that’s six.” Ex. 2030, 163:23–164:1.

Dr. Sharman states the Specification at column 24 “discloses a small number of alternative dose amounts that can be added to or replace the dosing in Figure 2.” Ex. 2008 ¶ 131. Dr. Sharman states

the specification teaches that “the amount of GDC-0199 in the initial doses administered to the patient can, for example, be between 20 mg to 60 mg (e.g., 20 mg, 25 mg, 30 mg, 35 mg, 40 mg, 45 mg, 50 mg, 55 mg or 60 mg doses), followed by dose amounts of 100 mg, 200 mg, 300 mg or more of GDC-0199.” *Id.*, 24:55-60.

Ex. 2008 ¶ 132.

Dr. Sharman states “in my opinion, a POSA would have recognized that the inventors were in possession of dosing with the additional amounts recited in claims 2, 3, and 5—*i.e.*, 20 mg (before the 50 mg dose).” Ex. 2008 ¶ 143. Dr. Sharman states “[s]ince the specification identifies the initial, intermediate and final doses that alternatively can be used in the dose escalation provided in Figure 2 for treatment of CLL/SLL, this would have conveyed to a POSA that a dose escalation with these recited doses would be useful in treating CLL/SLL.” Ex. 2008 ¶ 143. Dr. Sharman states

I note that the specification discloses only seven exemplary “initial doses” below 50 mg, one of which is the 20 mg initial dose recited in claim 2. *Id.* (reciting “initial doses” of 10 mg, 20 mg, 25 mg, 30 mg, 35 mg, 40 mg, 45 mg). The patent as a whole,

and column 24 specifically, teaches me any one of these initial doses can be effectively used in the treatment method according to the claimed invention. In my view a POSA would have recognized the inventors possessed the claimed invention.

Ex. 2008 ¶ 144.

Dr. Sharman was asked “[s]o does Column 24 give you any guidance about which amounts can be substituted into that scheme and result in effective treatment?” Ex. 1034, 100:6–8. Dr. Sharman answered “[w]hen I look at this, there is a very narrow range of dosages from which to pick a starting dose, specifically between 10 to 80 milligrams, and then specifically laid out with the exact dosages of 20, 25, 30, 35, 40, 45, 50, 55, 60 milligram dosages.” Ex. 1034, 100:9–14. Responding to a question of “how would a POSA know which doses to select?”, Dr. Sharman stated “I find that this is a narrow range from which to choose dosages, and a POSA could pick based upon clinical experience with the medication.” Ex. 1034, 112:19–23.

b. Descriptive support analyzed in view of intrinsic '942 Specification, expert declarations, and caselaw

Claim 2 depends from claim 1. It requires “a dose of 20 mg of GDC-0199 per day for one week” in addition to the doses recited in claim 1 (i.e., “50 mg of GDC-0199 per day for one week followed by a dose of 100 mg of GDC-0199 per day for one week”). As explained above, we find the Specification sufficiently supports the dosing regime recited in claim 1.

Patent Owner relies on the disclosures in Figure 2 and column 24 of the '942 patent as support for the addition of 20 mg of GDC-0199. Figure 2 shows administering GDC-0199 in escalating doses, with 50 mg as the starting dose, followed by 100 mg, and ends with 300 mg ending dose. *See*

Ex. 1001, Fig. 2. Column 24 discloses “the amount of GDC-0199 in the initial doses administered to the patient can, for example, be between 20 mg to 60 mg (e.g., 20 mg, 25 mg, 30 mg, 35 mg, 45 mg, 50 mg, 55 mg *or* 60 mg doses).” Ex. 1001, 24:55–59 (emphasis added). Dr. Sharman states “any one of these initial doses can be effectively used in the treatment method according to the claimed invention.” Ex. 2008 ¶ 144. We understand Dr. Sharman as stating that the initial doses disclosed in column 24 may be substituted for one another, and we agree that the ’942 patent treats 20 mg and 50 mg as alternative initial doses.

Dr. Sharman also repeatedly testifies that the ’942 patent discloses *multiple* “initial doses.” *See* Ex. 2008 ¶ 153 (“column 24 discloses that the multiple ‘initial doses’ of GDC-0199 can ‘be between 20 mg to 60 mg (e.g., 20 mg [and] 50 mg . . .)’” (citing Ex. 1001, 24:48–60, brackets added by Patent Owner)); ¶ 157 (“Column 24 discusses the dose escalation of Figure 2 and then discloses that the dose amounts of GDC-0199 in the dose escalation of Figure 2 can be changed to include multiple ‘initial amounts’ of ‘between 20 mg to 60 mg (e.g., 20 mg [and] 50 mg . . .)’” (citing Ex. 1001, 24:48–52, 55–60, brackets added by Patent Owner)). The Specification, however, does not support Dr. Sharman’s testimony. Indeed, instead of the conjunctive “and” added in the brackets by Patent Owner, the Specification, in fact, discloses 20 mg and 50 mg as alternatives with the disjunctive “or.” *See* Ex. 1001, 24:57–59.

Our esteemed colleague in the dissent finds persuasive Dr. Sharman’s statement that the doses listed in column 24 can be “*added to* the amounts taught in the Figure 2 exemplary dose escalation.” *See infra* dissent (citing Ex. 2008 ¶ 141 (emphasis added by dissent)). Although the Specification

may support the addition of intermediate and/or final doses, it does not support the addition of other initial doses, to the dosing regime disclosed in Figure 2. *See* Ex. 1001, 24:48–60.

We acknowledge the '942 patent uses the term “initial doses” in the plural form. *Id.* In our view, the plural “doses” refers to the multiple administrations in the initial cycle, and not multiple initial dosing amounts. The Specification supports our reading here. For example, the '942 patent discloses “FIG. 2 for an exemplary dosing scheme where amounts of GDC-0199 administered to patients in the first dosing cycle escalate from 50 mg doses the first week, to 100 mg doses the second week, to 300 mg doses the third week.” *Id.* at 24:37–41 (emphases added). In each instance, the '942 patent uses the plural “doses” to refer to the multiple dosing of a single amount.

In addition, logically and grammatically, there cannot be more than one *initial* dose amount. *See* Ex. 3001 (Merriam-Webster defining “initial” as “of or relating to the beginning: INCIPIENT” or “placed at the beginning: FIRST”). Thus, if, for example, 20 mg is the initial dose, then 50 mg cannot also be an initial dose but rather would be an intermediate or final dose. Here, Patent Owner does not argue, and Dr. Sharman does not point to any evidence to support, the addition of 50 mg as an intermediate dose.

The dissent states there are “generic disclosures in the '942 patent that support the use of 20 mg and 50 mg in a single escalating dose series.” *See* dissent. Specifically, the dissent finds “an ordinary artisan would recognize selection of 20 mg as an initial value, but then selecting any dose between 20 mg and 800 mg as an intermediate dose, and would especially recognize 50 mg as such a dose.” *Id.*

This is another instance where we disagree with our dissenting colleague. Factually, Patent Owner does not argue 50 mg is an intermediate dose. Instead, Patent Owner, relying on Dr. Sharman's testimony, argues 50 mg is the second "initial dose," in addition to the 20 mg initial dose. Legally, we emphasize that "[i]t is not sufficient for purposes of the written description requirement of § 112 that the disclosure, when combined with the knowledge in the art, would lead one to speculate as to modifications that the inventor might have envisioned, but failed to disclose." *Lockwood v. Am. Airlines, Inc.*, 107 F.3d 1565, 1572 (Fed. Cir. 1997).

Here, although the specification of the '942 patent may be understood to provide blazemarks to specific initial dose values, we find no blazemarks for selecting two different initial doses in a single treatment regime. Even if it would have been obvious to select two initial doses for an escalating dosing cycle of GDC-0199, "a description that merely renders the invention obvious does not satisfy the [written description] requirement." *Ariad*, 598 F.3d at 1352.

In sum, after reviewing the record, including the statements of Dr. Gordon and Dr. Sharman, we agree with Petitioner that a preponderance of the evidence on the entire record shows claim 2 lacks written descriptive support.

F. Written Description—Claims 3, 5, 14, 26, 27, 30

Claim 3 depends from claim 2, claim 5 depends from claim 3, claims 14 and 26 are independent claims and claims 27 and 30 depend from claim 26. Each of these claims, like claim 2, includes at least one additional dose of GDC-0199. Claims 3, 5, 14, and 27 recite multiple additional doses of

GDC-0199. Claims 26 and 30 encompass dosing intervals that differ from those recited in claim 1.

1. *Petitioner's position*

Petitioner contends:

The claims recite a precisely picked array of escalating doses: 50 mg and 100 mg in claim 1, adding a 20 mg cycle in claim 2, 200 mg in claim 3, and 400 mg in claim 5—each for “one week.” Claim 14 requires *all* these doses in distinct cycles; claim 26 requires just 50 mg, 100 mg, and 400 mg, with 20 mg reemerging in claim 27.

Pet. 43. Petitioner contends “the full combination of 20, 50, 100, 200, and 400 mg is nowhere to be found.” Pet. 43.

Petitioner contends “the set of possible dosage regimes made by all the specification’s lists is a massive genus, and Patent Owner later tried to claim advantageous species of that genus without describing them up front or providing guidance to their selection. Ex. 1020 ¶ 139.” Pet. 45. Petitioner contends “the skilled artisan would need to decide on doses, picking 20, 50, 100, 200, and 400 mg as claimed—while ignoring others from the list. *See* Ex. 1001, 4:34-37, 24:48–60 (providing laundry list of doses); Ex. 1020 ¶ 134.” Pet. 47. Petitioner contends the “specification provides no ‘guidance’ on how or why one would pick these doses, in this order, for these time periods, in the absence of an antibody, to fight a cancer that the specification takes pains to explain is antibody-prone. Ex. 1020 ¶ 135.” Pet. 47–48. Petitioner contends “even if a skilled artisan could fill in the blanks, ‘a description that merely renders the invention obvious does not satisfy the requirement.’” *Ariad*, 598 F.3d at 1352.” Pet. 48.

2. *Patent Owner's position*

Patent Owner contends “[t]hese claims find support in the specification, starting from the dosing disclosed in Example 2 and Figure 2, coupled with the specification’s teaching of alternative dose amounts to the dosing of Example 2 and Figure 2. Ex. 2008 ¶¶ 66-67, 137–148, 169.” PO Resp. 29. Patent Owner contends that

[a] POSA would have recognized that the starting, intermediate, and ending dosing of Example 2 and Figure 2 provides a framework for treating CLL/SLL with GDC-0199, and that column 24 discloses alternative GDC-0199 dose amounts that can be effectively used in this framework, leading to the dosing of the challenged claims. Ex. 2008 ¶¶ 140-147. Column 24 discloses that “the amounts of GDC-0199 per dose administered to the patient are increased during a first dosing cycle.” Ex. 1001, 24:34-37. Column 24 then refers to the schedule of Example 2 and Figure 2 as an example of this step-up dosing. *Id.*, 24:37-41 (“See, e.g., FIG. 2 for an exemplary dosing scheme . . .”). Column 24 goes on to disclose a small number of alternative dose amounts.

PO Resp. 30.

Patent Owner contends “to the extent that Petitioner contends that the specification must provide ‘blaze marks’ to meet the written description requirement (a contention with which Patent Owners disagree), that test is also met here.” PO Resp. 32. Patent Owner contends

Column 24 calls out Figure 2 as an “exemplary dosing scheme” and guides a POSA to the alternative doses (also listed in column 24) that can be used in this dosing scheme. *Id.* Specifically, column 24 refers to the dose escalation of Figure 2 and, in the same section, directs a POSA to a small number of additional doses that can be added to or replace the doses in Figure 2. *Id.* Those additional doses expressly include each of the claimed dose amounts in claims 2, 3, and 5.

PO Resp. 33. Patent Owner similarly contends for claims 14, 26, and 27 that the “limitations of each of these claims are supported by express disclosures of the specification for the same reasons as described above with respect to claims 2, 3, and 5. Ex. 2008 ¶¶ 66-67, 149–158, 169.” PO Resp. 34.

Patent Owner contends “Column 24 of the ’942 patent discloses a small number of doses from which a POSA would have recognized that the inventors had possessed the claimed methods using the recited doses and combinations. *Id.* ¶¶ 167-8.” PO Resp. 38. Patent Owner contends “Petitioner has pointed to no case law that supports a requirement that the specification list out every possible combination of the listed doses to establish written description support.” PO Resp. 39.

3. *Analysis*

a. *Experts’ views of descriptive support for claims 3, 5, 14, 26 27, and 30 in ’942 patent*

Dr. Gordon states

The claims require specific escalating doses: 50 mg and 100 mg in claim 1, adding a 20 mg cycle in claim 2, 200 mg in claim 3, and 400 mg in claim 5—each for “one week.” Claim 14 requires all these doses. Claim 26 requires just 50 mg, 100 mg, and 400 mg. These all appear in a particular order. In short, the combination of these particular doses in these order for these times is not disclosed anywhere in the specification.

Ex. 1020 ¶ 123. Dr. Gordon states “[t]here is nothing in the specification that provides any guidance to make this particular combination out of all the possible combinations, or suggests that the inventors actually possessed this precise combination.” Ex. 1020 ¶ 135.

Dr. Gordon states:

When you consider all the possible starting doses, ending doses, orders, and timings, the set of possible dosage regimes made by

all the specification's lists is a massive set. And I do not see adequate guidance within the specification to show that the patent applicants described principles of which ones from those combinations would work to be efficacious or to avoid TLS or other concerns.

Ex. 1020 ¶ 139.

Dr. Gordon was asked “Can we go back to Column 24? So I'm looking back again at lines 48 through 60. Do you agree that this expressly lists just 16 doses?” Ex. 2030, 171:4–8. Dr. Gordon stated:

No, I don't agree with that statement. That's a generalization. . . . there's quite a bit of a range, even though later on in that paragraph there are some examples given, those are just examples, but that doesn't mean it's an all-exclusive example, especially when there's discussion of ranges. . . . So there's potentially a lot more than 16 doses if one reads the text of that paragraph.

Ex. 2030, 171:9–172:19.

Dr. Sharman states “I disagree with Dr. Gordon's arguments that the exact combinations are not taught in the specification, *id.* ¶ 135, and that the specification discloses a “*fixed-dose* aspect of GDC-0199 administration,” *id.* ¶ 134.” Ex. 2008 ¶ 165. Dr. Sharman states

a POSA would have recognized that the dosing in Example 2 and Figure 2 is “exemplary,” but that the doses in Example 2 and Figure 2 are not the only doses to be used. Specifically, column 24 calls out Figure 2 as an “exemplary dosing scheme” and guides a POSA to the alternative doses (also listed in column 24) that can be used in this dosing scheme. *Id.*, 24:34-41, 24:48-60.

Ex. 2008 ¶ 133. Dr. Sharman states “column 24 discloses that the multiple ‘initial doses’ of GDC-0199 can ‘be between 20 mg to 60 mg (e.g., 20 mg [and] 50 mg . . .),’ ‘followed by dose amounts of 100 mg [or] 200 mg,’ with a ‘final amount[] of . . . 400 mg.’ *Id.*” Ex. 2008 ¶ 153. Dr. Sharman states

that as “a clinician and a POSA at the time, given the teachings of Figure 2, I would have found these to be acceptable options for use in a dose escalation treatment method.” Ex. 2008 ¶ 153. Dr. Sharman states “[i]n linking the dose escalation of Figure 2 to the additional doses in column 24, and given the Examples that teach independent activity with GDC-0199, the specification provides ‘blaze marks’ that guide me to the claimed invention.” Ex. 2008 ¶ 154.

Dr. Sharman was asked “[h]ow many possible combinations of 16 dose amounts are there in dose escalation treatment or cycle?” and answered “I think that the purpose here is that a dose escalation is safe and effective. It’s the totality of therapy that is safe and effective, so the number of possible combinations isn’t one I can calculate currently.” Ex. 1034, 146:14–22.

b. Descriptive support analyzed in view of intrinsic ’942 Specification, expert declarations, and caselaw

In the Institution Decision, we provisionally determined that claims 3, 5, 6, 14, 26, 27, and 30 lacked descriptive support, and the additional evidence in the Sharman Declaration does not persuade us otherwise. As we analyze these claims, we find support in *Schaumann* for the logic that selection of multiple species is not immediately envisaged and therefore reasonably is not described:

Even if we take the 10 examples of the French or the 12 examples of the Swedish reference, take them apart and recombine them into different compounds than those named, we do not get a small recognizable class with common properties. We would apparently get from the French patent some 130 and from the Swedish some 156 compounds. And in doing this we are not dealing with such closely related units as the H and CH 3

and the five hydroxyalkyl components in *Petering* but with such widely differing R 1 choices as p-acylaminobenzene, diphenyl, B-naphthalene and dimethylbenzene, to name a few from the thirteen possible choices. And for the R 3 choices there are such diverse radicals as ethyl, dodecyl, benzyl, and B-naphthyl. We will not apply the *Petering* type of analysis to such a situation.

Schaumann, 572 F.2d at 316.

We note regarding claim 26 that the '942 patent requires not only selection of the 400 mg dose but also selection from a large number of possible dosing periods:

A dosing period as used herein is meant a period of time, during which each therapeutic agent has been[] administered at least once. A dosing cycle is usually about 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, or 30 days, and, in one embodiment, 6, 7, 8, 9, 10, 11, 12, 13, or 14 days, for example, 7 or 14 days. In certain embodiments, a dosing period is a dosing cycle.

Ex. 1001, 17:29–38. Claim 26 administers GDC-0199 doses for 400 mg for an undefined period and therefore fails to satisfy the description requirement. And while we acknowledge that Dr. Sharman asserts that the description in column 24 of the '942 patent is sufficient to select any combination of doses of GDC-0199, we do not, as a matter of law, find this persuasive.

Specifically, Figure 2 shows administration of three doses of GDC-0199 of 50 mg for a week, 100 mg for a week, and 300 mg for a week. Selection of one additional dose for administration with the three recited doses in Figure 2 from the list in column 24 is reasonably understood as a selection from a genus of at most thirteen recited choices of 10 mg, 20 mg, 25 mg, 30 mg, 35 mg, 40 mg, 45 mg, 55 mg, 60 mg, 80 mg, 190 mg, 200 mg, and 400 mg. *See* Ex. 1001, 24:48–60. However, selection of two

additional doses for administration with the three recited doses in Figure 2 would result in a choice from thirteen recited doses multiplied by a choice from twelve recited doses, resulting in 156 possible combinations of doses.

While each of these 156 possible dose combinations may be obvious, “a description that merely renders the invention obvious does not satisfy the requirement.” *Ariad*, 598 F.3d at 1352. And *Schaumann* found that genus sizes of approximately 150 was insufficient to establish anticipation, supporting the reasoning that while such selections might, in some situations, be obvious, they do not satisfy the anticipation and analogous description requirement. *Novozymes* found such selection insufficient where the patent “provides formal textual support for each individual limitation recited in the claims . . . [but] nowhere describes the actual functioning [invention] . . . that those limitations together define.” *Novozymes A/S v. Dupont Nutrition Biosciences APS*, 723 F.3d 1336, 1349 (Fed. Cir. 2013). Instead, claims 3, 5, 6, 14, 25–27, and 30 seek “to derive written description support from an amalgam of disclosures plucked selectively from the” ’942 patent. *Id.* at 1349.

We are not persuaded that the sort of picking and choosing required to obtain two or more different dose amounts used in each cycle would “clearly allow persons of ordinary skill in the art to recognize that [the inventor] invented what is claimed.” *Ariad*, 598 F.3d at 1351. “Although this case [*Purdue*] differs from *Ruschig* in that what was disclosed in *Ruschig* was a genus encompassing potentially half a million compounds, the rationale applies equally to this case, in which the . . . patent discloses a multitude of pharmacokinetic parameters.” *Purdue Pharma L.P. v. Faulding Inc.*, 230

F.3d 1320, 1326 (Fed Cir. 2000) (*citing In re Ruschig*, 379 F.2d 990 (CCPA 1967)).

Claim 14, in particular, recites a GDC-0199 dosing regimen “compris[ing] a dose of 20 mg per day for one week, a dose of 50 mg per day for one week, a dose of 100 mg per day for one week, a dose of 200 mg of GDC-0199 per day for one week, and a dose of 400 mg per day.” Unlike for claim 1, such disclosure is not found in a single example, but results from picking and choosing three separate selections from column 24. “The written description requirement is not met when, as here, the specification provides at best disparate disclosures that an artisan might have been able to combine in order to make the claimed invention.” *Flash-Control, LLC v. Intel Corp.*, 2021 WL 2944592 *4 (Fed. Cir. 2021) (*citing Ariad*, 598 F.3d at 1352.)

We therefore find persuasive Petitioner’s argument that “the set of possible dosage regimes made by all the specification’s lists is a massive genus, and Patent Owner later tried to claim advantageous species of that genus without describing them up front or providing guidance to their selection. Ex. 1020 ¶ 139.” Pet. 45.

Accordingly, we find that Petitioner shows, by a preponderance of the evidence, that claims 3, 5, 14, 26, 27, and 30 of the ’942 patent are unpatentable as failing to comply with the written description requirement.

G. Written Description—Claims 6 and 25

Claims 6 and 25 depend from claims 1 and 14, respectively, and further require that “administration of GDC-0199 to the patient results in reduced tumor burden to the patient with acceptable toxicity.”

1. *Petitioner's Position*

Petitioner contends “Patent Owners cannot dispute that their patent does not explicitly disclose reducing tumor burden *See* POR 45-48.” Pet. Reply 22. Petitioner contends “Patent Owners also fail to argue that the patent discloses using an escalating-dose GDC-0199 monotherapy to shrink tumors. They can only point to negative controls in examples showing less tumor growth. POR 45-46.” Pet. Reply 22. Petitioner contends

Patent Owners also suggest that “acceptable toxicity” is disclosed because a POSA would know it means that “side effects should be proportional to risk of the disease.” POR 48. But the patent itself says nothing about this. Dr. Sharman only testified that a POSA would want to balance efficacy and side effects. POR 48; Ex. 2008 ¶¶ 179-80. Again, that is obviousness. Pet. Reply 23.

2. *Patent Owner's Position*

Patent Owner contends “Example 1 of the '942 patent, the results of which are shown in Figure 1 . . . shows that GDC-0199 alone has independent activity in reducing tumor burden as compared to the control.” PO Resp. 45. Patent Owner contends “[w]ithin 3 weeks of treatment, administering GDC-0199 alone led to 80% tumor growth inhibition relative to placebo.” PO Resp. 46. Patent Owner contends a “POSA would also have recognized that Example 4 specifically states and demonstrates that ‘single-agent treatment with GDC-0199 following combination with GA101’ ‘resulted in a significant delay in tumor growth’ and thus ‘sustains efficacy in vivo.’ Ex. 1001, 28:13-29; Ex. 2008 ¶ 175.” PO Resp. 47. Patent Owner contends:

As Dr. Sharman explains, it would have been well known to a POSA that side effects should be proportional to risk of the

disease. [Ex. 2008] ¶ 179. A POSA would have wanted the likelihood of benefit to outweigh the likelihood of risk, and would have understood that unacceptable toxicity occurs when side effects cause death, permanent injury or loss of organ function, or a similar serious adverse event which would not have occurred in the absence of treatment. [Ex. 2008] ¶ 180; Ex. 2025 at 13–26.

PO Resp. 48.

3. *Analysis*

a. Experts' views of descriptive support for claims 6 and 25 in '942 patent

Dr. Gordon states “I note that claims 6 and 30 recite a GDC-0199 monotherapy that ‘results in reduced tumor burden in the patient with acceptable toxicity.’ Nothing in the specification discloses this limitation.” Ex. 1020 ¶ 81. In his deposition, Dr. Gordon addressed reduced tumor burden and stated that “You can delay or inhibit tumor growth and not shrink a tumor back, you just can prevent it from getting bigger, which is different from shrinking a tumor back.” Ex. 2030, 20:6–9.

Dr. Gordon states:

“Acceptable toxicity” is only mentioned in Example 2, and even there it is only mentioned to propose that, during a combination therapy treatment, GDC-0199 will continue to be administered “*if they have acceptable toxicity.*” Ex. 1001 at 26:59-67 (emphasis added). Nowhere else is “toxicity” of GDC-0199 mentioned at all. Nor are criteria for what is “acceptable.” Overall, there is nothing demonstrating that the inventors actually possessed the claimed GDC-0199 escalating-dose monotherapy with acceptable toxicity.

Ex. 1020 ¶ 81. Dr. Gordon states “the specification does not explain what ‘toxicity’ means or how it would be “acceptable.” As a POSA, I understand this term to be a completely subjective judgment, as what is acceptable in

one treatment for one patient might not be acceptable for another.” Ex. 1020 ¶ 82.

Dr. Sharman states “[e]xample 1 of the ’942 patent shows that GDC-0199 alone has independent activity in reducing tumor burden as compared to the control. Specifically, in Example 1, the administration of GDC-0199 alone resulted in ‘demonstrated [tumor] stasis followed by tumor growth delay.’ Ex. 1001, 25:65-67.” Ex. 2008 ¶ 171. Dr. Sharman states that “[w]hile Example 1 and Figure 1 discuss non-Hodgkin’s lymphoma (‘NHL’), in my opinion, a POSA would have recognized from Example 1 that the effects of GDC-0199 alone would also demonstrate independent activity when used for the treatment of CLL or SLL.” Ex. 2008 ¶ 172.

Dr. Sharman states

The specification further states that “GDC-0199 monotherapy may be continued in patients beyond 6–7 cycles of combination treatment described above (e.g., if they have *acceptable toxicity* and have not yet achieved maximal clinical response (i.e., are having *continued improvement/reduction in tumor burden* that has not yet stabilized for at least 2 months)).

Ex. 2008 ¶ 177 (citing Ex. 1001, 26:59–64.) Dr. Sharman states “[f]rom the disclosure of monotherapy being continued with ‘acceptable toxicity’ and where the monotherapy is expected to result in ‘reduction in tumor burden,’ a POSA would have recognized that administration of GDC-0199 occurs with acceptable toxicity and some reduction in tumor burden, as recited in claims 6, 25.” Ex. 2008 ¶ 178. Dr. Sharman states “[a] POSA would have understood that ‘acceptable toxicity’ means the side effects of therapy do not outweigh the clinical balance of drug activity.” Ex. 2008 ¶ 179.

b. Descriptive support analyzed in view of intrinsic '942 Specification, expert declarations, and caselaw

Petitioner did not address claims 6 and 25 in the petition.⁴ The burden to show that these claims in the '942 patent are not described rests on Petitioner. *See Harmonic Inc. v. Avid Tech., Inc.*, 815 F.3d 1356, 1363 (Fed. Cir. 2016) (“[T]he petitioner has the burden from the onset to show with particularity why the patent it challenges is unpatentable.”)

[T]he hallmark of written description is disclosure. . . . the test requires an objective inquiry into the four corners of the specification from the perspective of a person of ordinary skill in the art. Based on that inquiry, the specification must describe an invention understandable to that skilled artisan and show that the inventor actually invented the invention claimed.

Ariad, 598 F.3d at 1351. Here, the Specification of the '942 patent teaches

GDC-0199 monotherapy may be continued in patients . . . if they have acceptable toxicity and have not yet achieved maximal clinical response (i.e., are having continued improvement/reduction in tumor burden that has not yet stabilized for at least 2 months)). Such patients may continue GDC-0199 monotherapy until they have achieved maximal response.

Ex. 1001 26:58–66. And while Dr. Gordon states the Specification lacks any disclosure of “reduced tumor burden” or “acceptable toxicity”, these two phrases are expressly recited in column 26 of the '942 patent. As to actual efficacy, Dr. Sharman points out that Figure 1 of the '942 patent demonstrates that treatment with GDC-0199 reduces the tumor burden in the NHL mouse model relative to the tumor burden present in mouse treated with saline. Ex. 2008 ¶ 171. We also credit Dr. Sharman’s understanding of

⁴ While we express concern that Petitioner did not address claims 6 and 25 with particularity in the Petition, the claim language and Specification of the '942 patent supports the patentability of these claims.

“acceptable toxicity” as balancing drug efficacy with side effects. Ex. 2008 ¶ 179.

Accordingly, we find that Petitioner does not show, by a preponderance of the evidence, that claims 6 and 25 of the ’942 patent are unpatentable for failing to comply with the written description requirement.

III. CONCLUSION

After considering the evidence and arguments presently before us in the complete trial record, we conclude that Petitioner has demonstrated, by a preponderance of the evidence, that Challenged Claims 2, 3, 5, 14, 26, 27, and 30 are unpatentable.⁵ We conclude that Petitioner has not demonstrated, by a preponderance of the evidence, that Challenged Claims 1, 6, and 25 are unpatentable.

In summary:

Claim(s)	35 U.S.C. §	Reference(s)/Basis	Claim(s) Shown Unpatentable	Claim(s) Not shown Unpatentable
1, 2, 3, 5, 6, 14, 25, 26, 27, 30	112(a)	Written Description	2, 3, 5, 14, 26, 27, 30	1, 6, 25

⁵ Should Patent Owner wish to pursue amendment of the Challenged Claims in a reissue or reexamination proceeding subsequent to the issuance of this decision, we draw Patent Owner’s attention to the April 2019 Notice Regarding Options for Amendments by Patent Owner Through Reissue or Reexamination During a Pending AIA Trial Proceeding, 84 Fed. Reg. 16,654 (Apr. 22, 2019). If Patent Owner chooses to file a reissue application or a request for reexamination of the challenged patent, we remind Patent Owner of its continuing obligation to notify the Board of any such related matters in updated mandatory notices. *See* 37 C.F.R. §§ 42.8(a)(3), (b)(2).

IV. ORDER

In consideration of the foregoing, it is:

ORDERED that claims 2, 3, 5, 14, 26, 27, and 30 of U.S. Patent 10,993,942 B2 are determined to be unpatentable; and

ORDERED that claims 1, 6, and 25 of U.S. Patent 10,993,942 B2 are not determined to be unpatentable; and

FURTHER ORDERED that, because this is a Final Written Decision, parties to the proceeding seeking judicial review of the decision must comply with the notice and service requirements of 37 C.F.R. § 90.2.

UNITED STATES PATENT AND TRADEMARK OFFICE

BEFORE THE PATENT TRIAL AND APPEAL BOARD

DR. REDDY's LABORATORIES, INC.,
Petitioner,

v.

GENENTECH, INC., HOFFMAN-LA ROCHE AG, and ABBVIE INC.,
Patent Owner.

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Patent 10,993,942 B2

FREDMAN, *Administrative Patent Judge*, Concurring-in-part, Dissenting-in-part.

I agree with the Majority decision and reasoning that a preponderance of the evidence supports Patent Owner's position that challenged claims 1, 6, and 25 have descriptive support in the '942 patent. I also agree with the Majority's position that a preponderance of the evidence supports Petitioner's position that claims 3, 5, 6, 14, 25–27, and 30 of the '942 patent are unpatentable as failing to comply with the written description requirement. My disagreement with the Majority solely focuses on claim 2 in the '942 patent.

A “disclosure need not recite the claimed invention *in haec verba*.” *Ariad*, 598 F.3d at 1352. The disclosure need only “clearly allow persons of ordinary skill in the art to recognize that the inventor invented what is

claimed.” *Id.* at 1351. In *Snitzer v. Etzel*, 465 F.2d 899, 902 (CCPA 1972), the court considered selection of an ion for use in a laser where the specification at issue “states that the fourteen expressly named ions may be used ‘separately or in various combinations.’” The court determined that “to approach this case realistically, the specification must be viewed as emphasizing the fourteen named ions. . . . and under these circumstances, we see no reason to hold that which is plainly described not to be described.” *Id.* at 903.

I recognize the Majority found that claim 2 lacks descriptive support in the Institution Decision, but this preliminary determination was made prior to Patent Owner’s submission of the Sharman Declaration. Ex. 2008. On this more complete record, I rely on Dr. Sharman’s statement that “in my opinion, a POSA would have recognized that the inventors were in possession of dosing with the additional amounts recited in claims 2, 3, and 5—*i.e.*, 20 mg (before the 50 mg dose).” Ex. 2008 ¶ 143. Dr. Sharman teaches that “the doses listed in column 24 can be substituted for or added to the amounts taught in the Figure 2 exemplary dose escalation.” Ex. 2008 ¶ 141. Dr. Sharman’s use of the word “added” and the plural term “doses” is reasonably understood as allowing for selection of two or more different initial doses. Dr. Sharman provides the reasoned logic for selection of one of these seven initial doses that “this is a narrow range from which to choose dosages, and a POSA could pick based upon clinical experience with the medication.” Ex. 1034, 112:19–23. Dr. Sharman states “[i]n my view a POSA would have recognized the inventors possessed the claimed invention.” Ex. 2008 ¶ 144.

I appreciate the Majority's reasoning that there cannot be two initial doses in a single escalating dose administration of GDC-0199. However, I am not persuaded by the Majority's reliance on the definition of the word "initial," particularly when the '942 Specification recites "initial doses." Ex. 1001, 24:56. There is no recited requirement that these "initial doses" are the same dose and the ordinary understanding of a plural term such as doses is reasonably understood to teach not only that multiple doses of the same amount may be selected, but that multiple doses selected from the immediately following list of seven recited "initial doses" may be selected.

Moreover, I am persuaded by Dr. Sharman's statement that the doses of column 24 can be "*added to the amounts taught in the Figure 2 exemplary dose escalation.*" Ex. 2008 ¶ 141 (emphasis added). There are also more generic disclosures in the '942 patent that support the use of 20 mg and 50 mg in a single escalating dose series such as the statement that GDC-0199

is administered each day of the dosing cycles of the multiple dosing cycles that follow the initial dosing cycle. The amount of GDC-0199 administered per day can be between about 10 mg to about 1,000 mg, about 20 mg to about 800 mg, about 20 mg to about 500 mg, or between about 50 mg to about 300 mg.

Ex. 1001, 4:32–37. That is, I find that an ordinary artisan would recognize selection of 20 mg as an initial value, but then selecting any dose between 20 mg and 800 mg as an intermediate dose, and would especially recognize 50 mg as such a dose because 50 mg was expressly listed as a dose choice at column 24, line 58 and was shown as a dose used in figure 2.

To the extent that the blazemark analysis of *In re Ruschig*, 379 F.2d 990 (CCPA 1967) applies here, the specification of the '942 patent provides blazemarks to the specific initial dose values recited in column 24 of the

Specification that would be immediately envisaged by the person of ordinary skill.

In my view, the immediately envisaged analysis of *In re Schaumann*, 572 F.2d 312, 316–317 (CCPA (1978)) applies here. In *Schaumann*, the CCPA found that in a disclosure which “embraces a very limited number of compounds closely related to one another in structure, we are led inevitably to the conclusion that the reference provides a description of those compounds just as surely as if they were identified in the reference by name.” In my view, that same inevitable reasoning applies here, where claim 2 represents a single selection from a list of seven specifically recited doses. And while *Schaumann* was not addressing written description, I would contend that anticipation and written description are opposite sides of the same coin and the same reasoning should apply to both as a matter of fairness.

I therefore agree with Patent Owner that a preponderance of the evidence of record—including the statements of Dr. Sharman—supports a finding of written descriptive support for claim 2 and that Petitioner did not meet their burden to show unpatentability.

Fundamentally, I do not take as constricted a view of the written description requirement as does the Majority and do not believe the caselaw requires that literal *ipsis verbis* is required for any claim amendment. I find the Specification would clearly allow persons of ordinary skill in the art to immediately envisage and recognize invention of single selection of a 20 mg dose in combination with the 50 mg and 100 mg escalating doses in Figure 2 of the '942 Patent for treatment of CLL/SLL as required by the specific embodiment of claim 2.

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